DRUGS · EAR PREPARATIONS

Steroid ear drops

Steroid-only ear drops treat inflammation without an antibiotic: eczematous and seborrhoeic otitis externa, the itchy canal, contact dermatitis from other drops, and canal granulation after the infection has been treated. Betamethasone and prednisolone drops are inexpensive and safe in the short term, and they are not ototoxic.

1 CHAPTER ONE At a glance

The drops on one page.

1.1 Summary

STEROID EAR DROPS

	SUMMARY
Preparations	Betamethasone sodium phosphate 0.1% drops (ear, eye and nose); prednisolone sodium phosphate 0.5%; steroid in combination with antibiotics is covered elsewhere
How they work	Reduce inflammation, oedema and itch by suppressing cytokines and inflammatory cell activity
ENT uses	Chronic eczematous, seborrhoeic and psoriatic otitis externa; allergic contact dermatitis of the canal (including from neomycin); granulation after treated infection; persistent itch
Dose	Betamethasone 2 to 3 drops every 2 to 3 hours initially, reducing as it settles (commonly three or four times daily for 7 to 14 days)
Avoid	Untreated bacterial, fungal or viral infection of the canal
Pregnancy	Short courses acceptable

2 CHAPTER TWO How they work

Mechanism and handling.

2.1 Mechanism

Topical corticosteroids act on keratinocytes and inflammatory cells to reduce cytokine release, vasodilatation and itch. They also reduce canal oedema, which reopens a narrowed canal.

3 CHAPTER THREE Using them in ENT

Indications and doses.

3.1 Chronic otitis externa

Treat the dermatosis: avoid scratching, cotton buds and water, keep the canal dry, use steroid drops or a steroid cream on a cotton bud for flares, and look for and treat co-existing fungal infection.

3.2 After infection

Once a bacterial otitis externa has been treated, a short course of steroid drops can help granulation and residual swelling settle.

4 CHAPTER FOUR Safety

What goes wrong.

4.1 Adverse effects

- Masking or worsening of **fungal** infection.

- Local skin atrophy with prolonged use.
- Rarely, sensitivity to excipients.

5 CHAPTER FIVE For the examination

What the examiner wants to hear.

5.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- The difference between infective and eczematous otitis externa and how treatment differs.
- Steroid drops as treatment for neomycin contact dermatitis.

5.2 Common pitfalls

COMMON PITFALLS

- Using steroid alone in an acutely infected, discharging canal.
- Ignoring a fungal infection that flares under steroid.

Written for this app from BNF and BNF for Children; NICE and specialist society guidance; Scott-Brown; Cummings. Figures reproduced from the books for private study. Study reference only: follow local protocols and senior advice for patients.